TRAUMA-08 | For therapists | Two separate age-based pathways

Integrating learning and planning next steps

1 | Purpose, fit and boundaries

Purpose of this unit

Gather learning, open goals and future support routes with the person. Consider expected changes and meaningful dates as needed; reaching unit 8 does not end treatment.

Choice and adaptation

Choose this sheet following professional assessment and with consent. The person may skip questions, answer aloud, or choose another tool. This is proposed original material for review; it provides no score, diagnosis, or promise of an outcome.

Boundaries specific to this unit

There is no recovery score, recurrence-prevention guarantee or fixed session count. Worsening or changed conditions require appropriate professional assessment. Do not automatically label renewed responses regression or lack of effort.

Reusing existing material

SES and MEANING may support summaries and goals. Unit 8 focuses continuation on trauma-related learning and needs; it does not replace outcome assessment or a treatment plan.

A fictional example for discussion

Fictional example: Yael notices progress but fears a period recalling the past. She and the therapist plan support and review timing, without treating fear as erasing learning or requiring a restart.

> Draft for administrator review. Clinical review and final rights approval are still pending.

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Working together with the worksheet

2 | Age-based pathways

CH / TE | Support within a TF-CBT pathway

CH/TE sheets summarize learning, support and future safety as established in treatment. Prepare child and supporter for change and renewed contact together, alongside professional needs assessment.

AD / OA | Support within a CPT pathway

AD/OA sheets summarize meanings, goals and remaining CPT topics. No success story or fixed session count is required; unit number does not determine ending.

Parallel work with a suitable support person

Agree with a suitable supporter how to listen, their role and the contact route. Support is not monitoring every thought. Adult involvement of others depends on choice and privacy.

Using W and P sheets

Choose the age version and relevant questions; completing every sheet is not a condition for progress. W accompanies professional conversation. P records only pre-agreed follow-up. Unit number does not mean session number.

Closing

Summarize what became clearer and what remains open. Use the existing SES-S01 and SES-S02 for the session framework. There is no need to repeat an assessment or plan already recorded.

Possible difficulties

If change is hard to see, revisit chosen goals and treatment information. Do not force a success story. New difficulty and continued-care needs belong in an accurate summary.

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Adapting for four age groups

3 | Age is a starting point, not an assumption about ability

CH | Children, approximately ages 6 to 11

CH expresses learning in the child's words; supporter and therapist remain responsible for support and contact. Do not make the child independently monitor deterioration or safety.

Offer one question at a time with little writing. Adult support must not replace the child's own voice.

TE | Adolescents, approximately ages 12 to 17

TE summarizes learning, setting changes and sharing limits while allowing independence. Age-group or setting transitions need planning, not blanket information sharing.

Agree in advance what will be shared and with whom, and explain the privacy limits in this setting. Do not promise absolute confidentiality.

AD | Adults

AD reviews meanings and stuck points within CPT and their life effects. Ending or continuing depends on assessment and needs, not completing every unit.

OA | Older adults

OA considers routine, health and support changes according to the person's choices. Plan accessible follow-up without assuming a companion must manage it.

Offer reading support, time, and conversation according to preference. Choose help from another person together with the participant, respecting their privacy.

> Each age version needs its own review. Approval of one version does not automatically approve the others.

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Practice, review and next steps

4 | Shared choice and learning from the attempt

Before offering practice sheet P

P records support and learning agreed in treatment. It sets no exposure, trauma narrative, dosage or component sequence. Without a plan, bring questions to the session. CH/TE sheets do not replace separate supporter work; AD/OA sheets are not official CPT materials.

At the next session

At the agreed time, review functioning, needs, learning and support. Jointly assess continuation, ending or a setting change. Keep the summary updateable; it is not automatic clinical approval.

Professional and technical records

Each version has a separate review record covering age suitability, originality, rights, and fictional filling. Reviewer and date fields remain blank until assigned. File checks or a code merge are not clinical approval. Clinical information stays only in the authorized setting, outside the code repository.

Background references

The following links were used for background only. Questions and adaptations were written afresh for the expansion plan; no questionnaire, scale, or protocol was copied. Adult sources do not validate the child versions. This material makes no claim about session count or treatment effectiveness.

Treatment-manual mapping status

Proposed: TF-CBT for children/teens; CPT for adults/older adults. Full manuals remain unmapped. Age, rights, supporter materials and separate-guide needs await review. Original support drafts, not an approved protocol.

www.nctsn.org/interventions/trauma-focused-cognitive-behavioral-therapy
tfcbt.org/about/
www.ptsd.va.gov/professional/treat/txessentials/cpt_for_ptsd_pro.asp

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